

GREENVUE AI · SERIES SEED 2026

The clinical **intelligence layer** for every diabetes clinic.

A proprietary AI platform — FHIR-native and CMS-agnostic — that turns routine diabetes visits into longitudinal intelligence. Built by an optical imaging team whose retinal AI is the core, not a feature.

Proprietary AI Engine

FHIR-Native

CMS-Agnostic

Seeking \$500K Seed

Live PoC Q3 2026

THE PROBLEM

Diabetes is **winning**. The clinics managing it are losing ground.

38M+

Americans with diabetes — 1 in 10 adults, growing every year

CDC · 2024

\$412B

Annual US cost of diagnosed diabetes — direct + indirect

ADA · 2022

50%

Of diabetic retinopathy cases undetected until vision loss begins

NEI · 2023

2–3 hrs

A full diabetes assessment takes — yet only 80–110 min is clinical. The rest: wait and paperwork

Clinic workflow data

The workflow problem

- Nurses manually transcribe all lab results into CMS after each investigation — error-prone, slow, a 5–15 min bottleneck before the physician can review
- Retinal images graded manually or sent to a remote grader — delays, inconsistency, missed early-stage DR
- Rich longitudinal patient data sits siloed in CMS, never synthesized — physicians make decisions without trend context
- Counselling is generic, 30–45 min, disconnected from today's lab results

The gap nobody has seized

- EHR giants (Epic, athenahealth) built record systems — not intelligence systems. They store data; they don't reason over it
- AI point solutions exist for retinal grading alone — without workflow integration they create more work, not less
- No platform stitches together the full visit — intake to referral — with AI woven through every step
- LLM + AI orchestration technology only became capable enough to solve this in the last 18 months

WHY NOW

Three forces converged. The build window is open right now.

1 · LLMs can now reason clinically

State-of-the-art language and vision models can process lab trends, analyze medical images, and generate physician-quality clinical summaries. Two years ago this wasn't reliable at clinic-scale cost. Today the economics work — and the accuracy is there.

2 · FHIR is now law in the US

The 21st Century Cures Act mandates FHIR APIs for all certified US EHRs. Epic, athenahealth, Cerner — all legally required to expose a standard API. A FHIR-native AI layer connects to the entire US market without clinic-by-clinic custom integration. The regulatory tailwind arrived in 2022.

3 · AI orchestration platforms are production-ready

Coordinating multiple specialized AI components across a full clinical workflow was a research problem in 2023. In 2026 it is a product sprint. The infrastructure exists. The expertise to build it is the differentiator.

"The window where this is buildable but not yet built is open right now."

Healthcare AI investment hit \$6.1B in 2024 — but overwhelmingly into hospital-scale infrastructure and narrow diagnostic tools. The intelligence layer for mid-size specialty clinics remains wide open.

US HEALTHCARE AI MARKET

\$187B by 2030

Grand View Research · 14.4% CAGR

OUR SOLUTION

GreenVue Clinical AI Engine. Sits above your CMS. Replaces nothing. Transforms everything.

YOUR INTERFACE

GreenVue AI Dashboard

Physician + nurse-facing interface. Surfaces clinical summaries, risk flags, referral recommendations, and care briefs. The only new thing clinic staff interact with.

Web UI · Any device



OUR IP ★

GreenVue Clinical AI Engine

Proprietary multi-component AI orchestration platform. Handles intake intelligence, clinical decision support, real-time retinal analysis, automated data capture, patient engagement, and population health — all coordinated through a single intelligent layer. Built on a decade of optical imaging R&D.

Proprietary · Protected



STANDARD PROTOCOL

FHIR / HL7 Adapter Layer

Connects to any CMS via FHIR R4 — mandated for all US certified EHRs. Device integration via HL7 and DICOM standards. One architecture covers Epic, athenahealth, Cerner, and hundreds of smaller systems.

CMS-Agnostic



UNTOUCHED

Existing CMS / EHR — any vendor

Epic · athenahealth · Cerner · proprietary clinic software. No migration. No retraining. No data loss. Zero displacement risk — the clinic keeps everything they have.

Preserved as-is

Key capabilities: Real-time retinal image analysis · Longitudinal clinical risk scoring · Zero-friction device-to-record data capture · Personalized patient engagement · Population-level health intelligence

ECONOMIC IMPACT

50 patients per day. Here's what GreenVue AI delivers in dollars and minutes.

50

patients / day

~\$270K+

annual economic value created

\$1,250

monthly subscription

>200x

ROI on annual subscription

Not captured: Google review uplift from faster, better visits — the #1 driver of new patient acquisition for independent diabetes clinics.

⚠ Without GreenVue AI

Manual data entry lag

15 min

Per patient — nurse transcribes all results into CMS. 50 patients = 12.5 hrs nurse time lost daily.

Retinal grading turnaround

24–72 hrs

Images sent to remote grader. Findings not available at same-visit final consult.

Missed screening revenue

~\$18K/mo

30% of at-risk patients not screened due to workflow friction. \$120/screening × gap.

Clinical decisions

Recall-based

No longitudinal trend surfaced automatically. HbA1c trajectory, risk scores — physician carries this mentally.

✓ With GreenVue AI

Data entry lag

0 min

Device-to-record direct capture. Results appear in real time. Annual nurse productivity saving: ~\$45K.

Retinal analysis

<2 min

AI grades fundus image during the visit. Severity, urgency flag, and prior-scan comparison ready for final consult.

Recovered screening revenue

+\$18K/mo

System flags every overdue screening at intake. Workflow friction eliminated. Completion rate rises to near 100%.

Clinical decisions

Data-driven

HbA1c trend, DR progression, CVD risk score surfaced before physician begins. Consistent referral thresholds every visit.

→
AI
LAYER

MARKET SIZE

A massive, underpenetrated market. **We enter focused and expand with proof.**

\$14.2B **TAM** — US diabetes care management software market · 2030 projection

\$2.8B **SAM** — Independent diabetes + endocrinology clinics + primary care with diabetes panels · AI-ready subset

\$140M **SOM · Year 3** — 500 US clinics at \$1,250–2,500/mo · conservative 5% SAM penetration

Beachhead: India PoC → US entry

- Live build partner: diabetes specialty clinic — 50+ patients/day, same-visit comprehensive assessment
- India as engineering sandbox: lower cost, faster iteration, real clinical data to validate AI platform
- US entry from Day 1 of product: FHIR-native architecture means no re-engineering for US EHRs
- Target beachhead: 7,000+ independent US endocrinology and diabetes specialty practices — underserved by Epic-scale solutions

Expansion vectors

- Cardiology clinics — same platform, different clinical logic module
- Nephrology — natural comorbidity with diabetes patient base
- Ophthalmology — retinal AI becomes a standalone referral tool
- Hospital networks — population intelligence scales to 10,000+ patient cohorts

UNFAIR ADVANTAGE

"We didn't add retinal AI to a CMS. We built the platform around the retinal AI."

10+ years of optical intelligence IP

- Magnas Inc. (8.5 yrs): image processing, color science, display calibration — commercialized into Samsung and LG pipelines
- GreenVue OCS/ColorPlus: optical calibration systems for display manufacturing — same image fidelity science applied to medical imaging
- Defense/airport AI: real-time object detection from optical sensor feeds — the same pipeline that powers our retinal analysis under clinical time pressure

This is not a pivot. This is the same IP in a new domain.

The data flywheel — the moat that compounds

- Every visit processed enriches the clinical AI models — risk stratification sharpens, referral thresholds calibrate better
- Clinic #50 gets a measurably smarter platform than clinic #1 got — automatically
- A competitor starting today will always be behind on clinical training data

Chief Scientist — clinical AI domain expertise

- Clinical domain expert driving the intelligence layer — the reasoning chains that make our platform clinically valid, not just technically functional
- This is our deepest moat: a generic software team can assemble AI components. They cannot replicate deep clinical AI prompt engineering built on real clinical experience

Clinical IP compounds with every patient visit

The founding team

- Bharat Anadure — CEO. EE + CS + MBA. Three founder roles across optical intelligence, ad-tech, and defense AI.
- Michael Anderson — CTO. Architect of GreenVue's optical and AI platforms. Leads platform engineering.
- Joseph Holmes — Chief Scientist. Clinical domain AI. The reason our platform reasons like a clinician.

COMPETITIVE LANDSCAPE

Nobody occupies our position. Here is why.

PLAYER	FULL-VISIT AI WORKFLOW	CMS-AGNOSTIC / FHIR	RETINAL AI BUILT-IN	CLINICAL INTELLIGENCE	INDIE CLINIC PRICE
GreenVue AI	✓ End-to-end	✓ FHIR R4 native	✓ Optical IP core	✓ Proprietary engine	✓ \$1-2.5K/mo
Epic / athenahealth	✗ Records only	They are the CMS	✗ None	✗ None	✗ \$500K+ deploy
Retinal AI point solutions	✗ Retinal only	Partial	✓ FDA cleared	✗ None	\$2-5K, narrow scope
Generic AI workflow tools	Partial	✗ CMS-replacing	✗ None	✗ Generic LLM	Varies
Hospital AI platforms	Hospital-scale only	Partial	✗ None	Partial	✗ \$1M+ enterprise

Our position: GreenVue AI is the only platform combining full-visit AI orchestration, FHIR-native CMS-agnosticism, optical-IP-driven retinal intelligence, and deep clinical reasoning — at a price point accessible to independent diabetes clinics. The EHR giants go upmarket. Point solutions stay narrow. We connect everything, for the clinics everyone else ignores.

BUSINESS MODEL

SaaS subscription. Revenue from day one of clinic deployment.

Tier 1 · Standard

\$1,250/mo

- ✓ Full AI platform access
- ✓ FHIR integration + 2 device connectors
- ✓ Up to 1,500 patient-visits/month
- ✓ Clinical dashboard + population reports

Independent diabetes / endo clinics

Tier 2 · Growth

\$2,500/mo

- ✓ Unlimited patient-visits
- ✓ All device connectors + custom FHIR mappings
- ✓ Multi-physician cohort dashboards
- ✓ Priority support + clinical customisation

Group practices · 3–10 physicians

Unit economics · 50-patient/day clinic

Monthly subscription	\$1,250
LLM + AI API costs	-\$363
Cloud infrastructure	-\$85
Support allocation	-\$50
Gross margin	\$752 · 60%

Revenue milestones

- **Month 6:** India PoC live — validation data collected
- **Month 12:** 10 paying clinics → \$12,500 MRR · \$150K ARR
- **Month 18:** 30 clinics → \$37,500 MRR · \$450K ARR · Series A

TRACTION & ROADMAP

Live build partner secured. Architecture underway. US entry in planning.

NOW · PHASE 0 Foundation Q2-Q3 2026	PHASE 1 India PoC Q3-Q4 2026	PHASE 2 US Entry Q1-Q2 2027	PHASE 3 Scale 2027-2028
→ Live diabetes clinic build partner (India) — workflow mapped, device inventory underway → LOI in progress — intent to deploy GreenVue AI as primary clinical intelligence layer → System architecture complete — FHIR-first specification → Clinical AI Engine scaffold in development → Seed round: \$500K target	→ CMS integration via FHIR adapter → Device connectors: retinal camera, POCT, ECG, ABI → AI retinal analysis live — validated vs. manual grading → Full platform running on real supervised patient visits → Clinical outcomes data: accuracy, time savings, physician NPS	→ India PoC data → US physician conversations and pilot outreach → FHIR R4 connector tested against US EHR sandboxes → 2-3 US independent diabetes/endocrinology clinic pilots → US regulatory position confirmed: clinical decision support framing → Series A preparation: \$1.5M raise	→ 30+ US clinic deployments — diabetes expanding to cardiology, nephrology → Multi-clinic config: deploy clinic #30 in days, not weeks → Hospital network pilots — population intelligence at scale → Retinal AI as standalone API — licensing to ophthalmology networks → Full team salaries + engineering team of 8
Milestone: Full platform demo on mock patient data	Milestone: 500 real visits processed · validation report	Milestone: First US paying clinic · \$150K ARR run-rate	Milestone: \$450K ARR · Series A close

USE OF PROCEEDS · \$500K SEED

Six months. One milestone. A live platform on real patients.

40%

\$200,000

Engineering & Development

Founding team stipends — Bharat, Mike, Joe at pre-revenue rates. Full-time focus for 6 months without burning runway on full salaries.

25%

\$125,000

Engineering Interns & Contractors

2-3 paid engineering interns for device connectors, FHIR adapter, and UI. Targeted contractor work for specialized integration components.

15%

\$75,000

AI API + Cloud Infrastructure

State-of-the-art LLM APIs for reasoning and vision analysis, plus cloud hosting with appropriate data residency for India PoC and US pilot.

10%

\$50,000

US Market Development

US clinic outreach — travel, demos, pilot conversations. Legal: IP protection, regulatory review, SaaS agreements. Relationship-driven BD, not marketing spend.

10%

\$50,000

Reserve / Operations

Buffer for timeline overrun, integration complexity, and clinical validation iteration. Healthcare software always has surprises. We budget for them.

What \$500K delivers

- Full AI platform running on live patients
- Clinical validation data from 500+ real visits
- FHIR R4 connector — US EHR compatible from day one
- The evidence package that closes the Series A

Series A target: \$1.5M · Q2 2027

Raising **\$500K** to prove clinical AI at scale.

\$500K

Seed round. Convertible note or SAFE. 6-month runway to live PoC and clinical validation data.

Targeting close Q3 2026

500+

Real patient visits on the platform before Series A. The clinical evidence that de-risks the next round.

India PoC → US pilot pipeline

\$1.5M

Series A target at Month 18. First US paying clinic. \$150K ARR. Full team salaries. 30-clinic rollout begins.

Q2 2027 · subject to milestones

*We are not building a CMS. We are building the **intelligence layer that makes every diabetes clinic smarter** — without asking them to change a single system they already use. The retinal AI at our core is not an add-on. It is the reason a decade of optical imaging experience pointed us here. **This is the software opportunity our hardware work was preparing us for.***